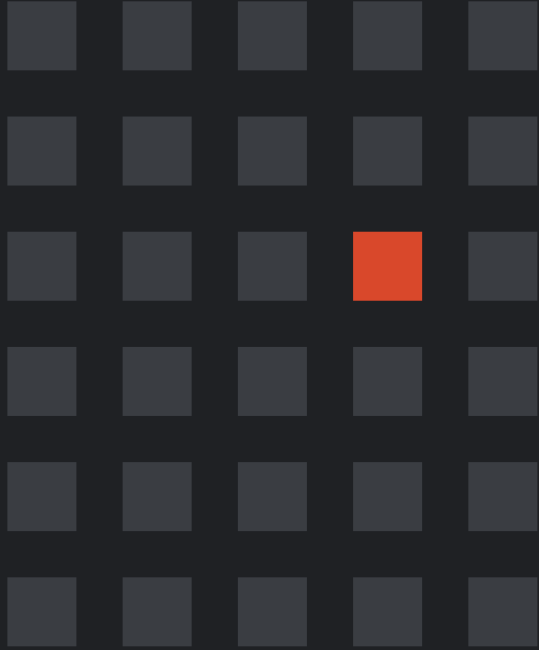




FREE PUBLIC TOOL / ARCHITECTURE BRIEF

GLP-1 Support: a free, private organizer

A browser-only notebook for people on, changing, or coming off GLP-1 medicines. It organizes; it doesn't advise.

Not medical advice. Not for emergencies.

It does not diagnose, does not give or calculate doses, and does not advise starting, stopping, or changing any medicine.

Emergency: call 911.

Personal project · Prepared October 6, 2026

Not medical advice. Sample data only. Personal project built on my own time. Not affiliated with or endorsed by my employer; views are my own.

The gap after the prescription

A lot happens between appointments.

INSTRUCTIONS

88%

said they got medication instructions (CAMP)

REFERRALS

27%

got a referral to any other service, such as nutrition counseling (CAMP)

COST

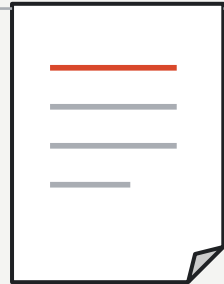
68%

said cost influenced starting or continuing (Navitus)

FILLING

1 in 3

had trouble filling a prescription, most often out of stock (GoodRx)



THE PRESCRIPTION

In the CAMP study

2,628 adults at one U.S. academic health system

88%

got medication instructions

27%

got a referral to any other service, such as nutrition counseling

37.7%

of those who stopped: insurance stopped covering the medicine, the most common reason

Sources: Tewksbury et al., *Frontiers in Nutrition*, Sept. 8, 2026 (CAMP, 2,628 adults) · Navitus Pulse Survey, Jan 2026 (2,000 users) · GoodRx Research via YouGov, Aug–Oct 2025 (585 users).

Six tools, one private notebook

- **Daily log** with a 7-day summary
- **Protein, fluids & fiber** checklist
- **Strength & muscle** tracker
- **Maintenance / coming off** planner
- **Insurance & refills** dates and questions
- **Questions for my prescriber**

01

Daily log

Sample: "Nausea: mild ·
Fluids: 6 cups." 7-day
summary.

02

Protein, fluids & fiber

Daily checklist with
general reference ranges,
"not a prescription."

03

Strength & muscle

Weekly tracker based on
the CDC adult guideline.

04

Maintenance / coming off

The plan agreed with the
prescriber, 30/60/90-day
check-ins.

05

Insurance & refills

Prior-auth and refill
dates. "A checklist, not
insurance advice."

06

Questions for my prescriber

Pick topics, then print or
download the list.

Shown with sample data only.

ALWAYS VISIBLE Not medical advice · Emergency: 911
Took too much: Poison Help 1-800-222-1222 · Crisis: call or text 988

It organizes, it never advises

01

Never advises

It does not diagnose, does not give or calculate doses, and does not advise starting, stopping, or changing any medicine.

Never points to sources of medication.

02

Sourced red flags

A red-flag card summarizing two Medication Guides (“not a complete list”), always visible.

The red-flag text is static and sourced.

03

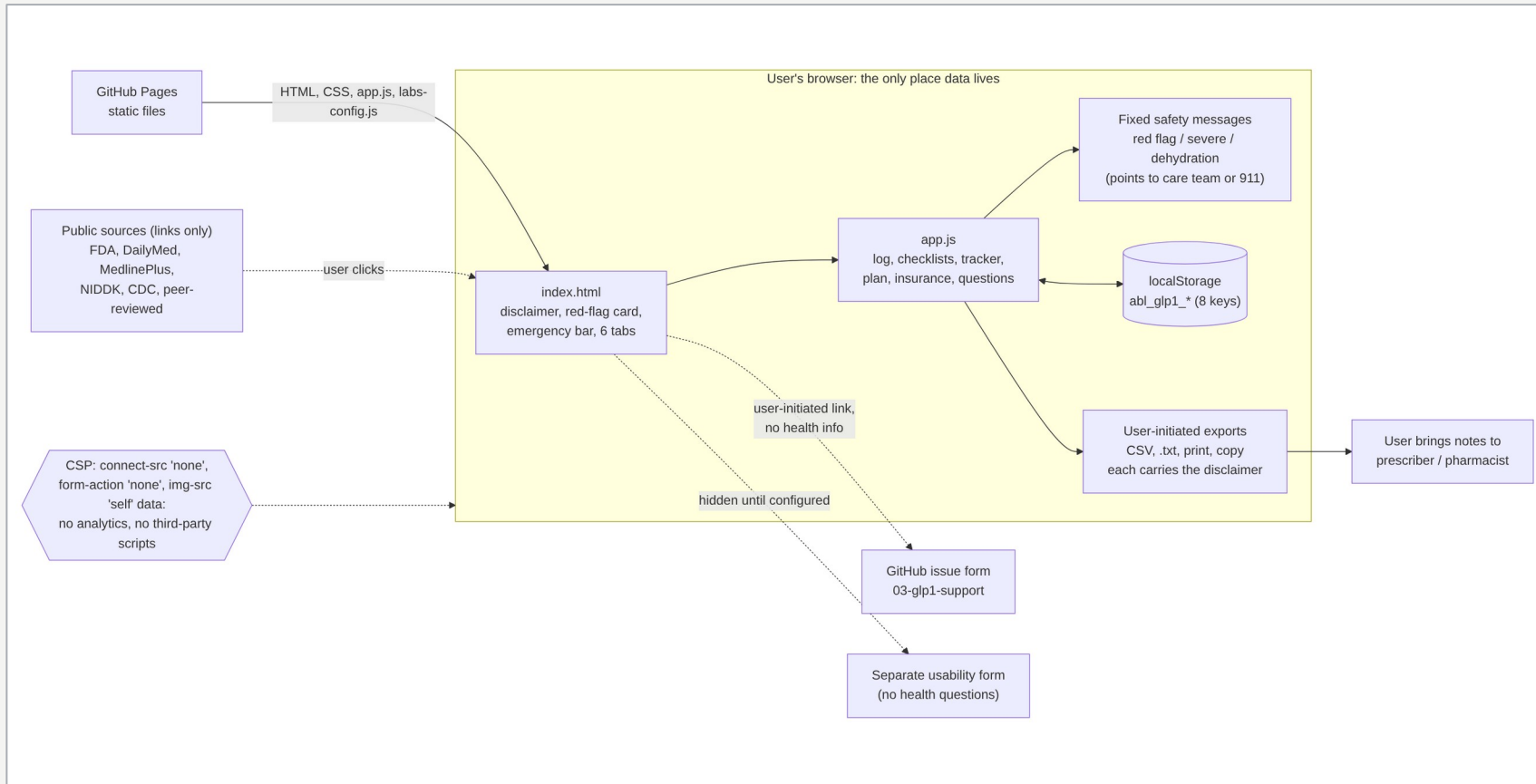
Fixed messages

On save, fixed messages point to the care team or 911 when a red flag is ticked, a symptom is marked severe, or vomiting or diarrhea is moderate or severe.

Emergency? Call 911. Took too much? Poison Help 1-800-222-1222. In crisis? Call or text 988.

“No alert doesn’t mean you’re okay.” “This tool can’t judge how serious a symptom is.”

Architecture: the browser is the only place data lives

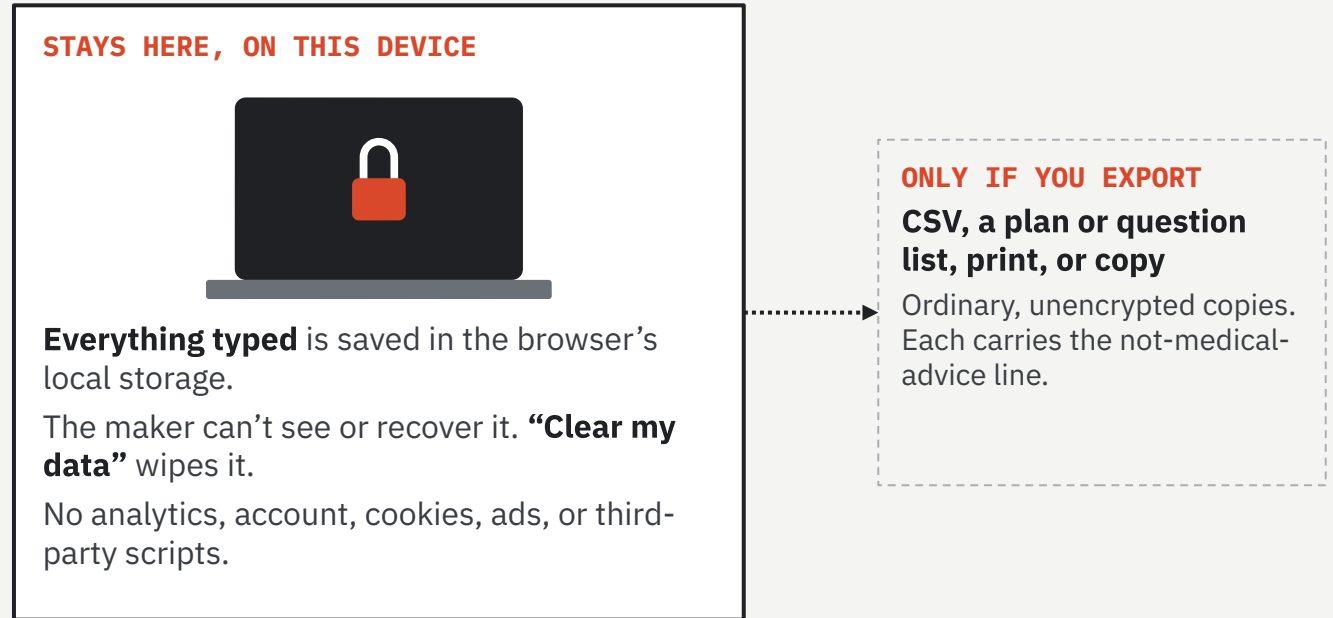


Read it left to right

- Static files on GitHub Pages and one script, app.js.
- Data sits in localStorage: abl_glp1_* (8 keys).
- The strictest CSP in Labs: connect-src 'none', form-action 'none'.
- No analytics script, no third-party code.

What leaves the browser: nothing the user types

- **No analytics, no account, no signup.**
- **Exports happen only when the user acts**, and each carries a disclaimer.
- **The author can't see or recover** what is saved.
- **The host logs visitors' IPs** for security, and the page says so.



Honest footnote: the page is hosted on GitHub Pages, which logs visitor IP addresses for security; the maker doesn't receive those logs. Not a medical record: not connected to any doctor, pharmacy, or health plan.

Governance: sourced, dated, reviewed

- | | | |
|----|---|------|
| 01 | Checked against FDA Medication Guides Content last reviewed September 30, 2026. Labels change, and the page says so. | DONE |
| 02 | Legal-risk memo fixes applied Wording, Terms, and export-disclaimer fixes (commit 913dbb9). | DONE |
| 03 | Bug form forbids health info It requires ticking “I did not include personal health information.” | DONE |
| 04 | Live page matches the commit index.html and app.js hash-checked against 4dd4953 on October 6, 2026. | DONE |
| 05 | Attorney consult still open The memo recommended one. Whether an attorney has reviewed the page is unknown. | OPEN |

Scale and limits: static files go a long way

~970K

uncached page loads a month fit under the cap (page weight about 103 KB)

100 GB

GitHub Pages soft bandwidth limit per month

0

data-breach surface: no backend, so no scaling cost

THE TRADE-OFFS

Data is per device: no backup and no cross-device use.

The help bot runs on a single box: one process, no failover.

Direction: stay local-only

Area	Next step	Will not do
Feedback	Set up the usability form (no health questions)	Add analytics
Content	Re-check DailyMed labels before wider promotion	Add dosing tools
Help bot	Refresh the bot FAQ	Add AI chat
Data	Keep every note on the user’s device	Add accounts or sync

Why: the legal-risk memo lists these features as ones that would raise risk.
Not medical advice. Emergency: call 911. Took too much: Poison Help 1-800-222-1222. Crisis: call or text 988.